

Women’s Health Requirements Discovery Pack

Current Understanding

Please review each line and confirm, correct, or add anything that is missing.

Prepared for requirement discussion.

Area	What we understood from your email and attached documents	Source	Agree as written? (Yes / No / Amend)	If not please correct or amend	Needed in phase 1? (Yes / No / Later)	Owner	Notes
Problem	Gynaecology grading currently requires looking across multiple sources such as clinic letters, discharge summaries, lab results, and radiology.	Email from Jasveen			Yes		
Problem	A one-page summary is the first practical step to reduce clicks and save grader time.	Email from Jasveen			Yes		
Problem	The longer-term goal is for the system to learn from clinician grading over time.	Email from Jasveen			Later		
Problem	Colposcopy may be easier to structure first because it is more category-based and requires less supporting information than gynaecology.	Email from Jasveen			Yes		
Current state	What system or tools does the service currently use to manage and grade referrals? (please describe)	To be confirmed by service			Yes		
Current state	How do referrals currently arrive at the service? (eFax, electronic referral, letter, other?)	To be confirmed by service			Yes		
Current state	Who currently grades referrals and what are their roles? (consultant, CNS, registrar, other?) How many people?	To be confirmed by service			Yes		
Current state	Approximately how many referrals does each service receive per week or per month?	To be confirmed by service			Yes		
Other vendor	Another vendor is also working on a related grading project. We need to understand what they are covering and where the boundary is between their work and ours.	Email from Jasveen			Yes		
Governance	Data sovereignty is important and may limit where data, documents, and AI processing can be hosted.	Email from Jasveen			Yes		
Governance	Access to the national colposcopy database (NCSR) is restricted and requires confidentiality and safety training.	Email from Jasveen			Maybe		
Colposcopy	Colposcopy grading should follow the attached colposcopy grading guide and the current grading template used by the service.	COLP Grading Guide + current grading template			Yes		
Colposcopy	Some colposcopy decisions may depend on previous national registry history rather than only the current referral.	Email from Jasveen			Maybe		
Colposcopy	A detailed extracted list of colposcopy booking-priority rules is now available from the scanned guide, but it still needs local clinical confirmation before being treated as final.	Detailed extraction prepared from scanned colposcopy guide			Yes		
Gynaecology	Gynaecology grading should follow the attached guideline categories and priority rules.	Gynaecology guideline PDF			Yes		
Gynaecology	AUB appears to require ultrasound and supporting clinical context before grading.	Gynaecology guideline PDF			Yes		
Gynaecology	PMB appears to require ultrasound and endometrial findings before grading.	Gynaecology guideline PDF			Yes		
Gynaecology	Fibroid grading appears to depend on size plus symptom burden.	Gynaecology guideline PDF			Yes		
Gynaecology	Ovarian or adnexal mass grading appears to depend on imaging features and tumour markers such as CA-125.	Gynaecology guideline PDF			Yes		

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Gynaecology	Pelvic pain grading appears to require more supporting context than a diagnosis label alone.	Gynaecology guideline PDF			Yes		
Gynaecology	Urogynaecology, fertility, PCOS, cervical polyp, paediatric gynaecology, pelvic tear, and tubal ligation are all in scope in the attached guideline.	Gynaecology guideline PDF			Yes		
Workflow	A clinician should remain able to confirm or change the final outcome.	Email plus service workflow assumption			Yes		
Workflow	Some referrals will need return to GP, reject, or request-for-more-information outcomes rather than direct booking.	Gynaecology guideline PDF + service workflow assumption			Yes		
Pilot	Validation should use redacted real referrals where possible.	Email from Jasveen			Yes		
Pilot	One service may be piloted before the other if that is simpler operationally.	Working assumption based on the email			Yes		
Pilot	What does the service consider a successful pilot? (please describe)	To be confirmed by service			Yes		